Cefuroxime: Drug information - UpToDate

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Contributor Disclosures

For additional information see ”Cefuroxime: Patient drug information” and ”Cefuroxime: Pediatric drug information”

For abbreviations, symbols, and age group definitions show table

Brand Names: Canada

- APO-Cefuroxime;
- Auro-Cefuroxime;
- Ceftin;
- JAMP-Cefuroxime

Pharmacologic Category

- Antibiotic, Cephalosporin (Second Generation)

Dosing: Adult

Dosage guidance:

Dosing: IM is an FDA-approved route and can be given at the same dose as IV.

Bite wound infection, prophylaxis or treatment

Bite wound infection, prophylaxis or treatment (animal or human bite) (alternative agent) (off-label use): Oral: 500 mg twice daily, in combination with an agent appropriate for anaerobes (Ref). Duration of prophylaxis is 3 to 5 days (Ref). Duration of treatment for established infection typically ranges from 5 to 14 days and varies based on clinical response and patient-specific factors (Ref).

Chronic obstructive pulmonary disease, acute exacerbation

Chronic obstructive pulmonary disease, acute exacerbation: Note: Avoid use in patients with risk factors for *Pseudomonas* infection or poor outcomes (eg, ≥65 years of age with major comorbidities, FEV_1 <50% predicted, frequent exacerbations) (Ref).

Oral: 500 mg twice daily for 5 to 7 days (Ref).

Intra-abdominal infection, mild to moderate, community acquired in patients without risk factors for resistance or treatment failure

Intra-abdominal infection, mild to moderate, community acquired in patients without risk factors for resistance or treatment failure (off-label use):

Cholecystitis, acute: **IV:** 1.5 g every 8 hours; continue for 1 day after gallbladder removal or until clinical resolution in patients managed nonoperatively (Ref). **Note:** The addition of anaerobic therapy (eg, metronidazole) is recommended if biliary-enteric anastomosis is present (Ref).

Other intra-abdominal infection (eg, appendicitis, diverticulitis, intra-abdominal abscess): **IV:** 1.5 g every 8 hours in combination with metronidazole. Total duration of therapy (which may include transition to oral antibiotics) is 4 to 5 days following adequate source control (Ref). For diverticulitis or uncomplicated appendicitis managed without intervention, duration varies from 4 to 14 days depending on disease extent, response, and immune status (Ref); for perforated appendicitis managed with laparoscopic appendectomy, 2 to 4 days may be sufficient (Ref).

Lyme disease

Lyme disease (*Borrelia* spp. infection):

Erythema migrans: **Oral:** 500 mg twice daily for 14 days (Ref).

Carditis (initial therapy for mild disease [first-degree atrioventricular block with PR interval <300 msec] or step-down therapy after initial parenteral treatment for more severe disease once PR interval <300 msec): **Oral:** 500 mg twice daily for 14 to 21 days (Ref).

Arthritis without neurologic involvement (alternative agent) (off-label use): **Oral:** 500 mg twice daily for 28 days (Ref).

Odontogenic soft tissue infection, pyogenic

Odontogenic soft tissue infection, pyogenic (initial therapy for mild infection or step-down therapy after parenteral treatment) (alternative agent) (off-label use):

Note: For patients unable to take penicillin (Ref).

Oral: 500 mg twice daily in combination with metronidazole; continue until clinical resolution, typically for 7 to 14 days. Use in addition to appropriate surgical management (eg, drainage and/or extraction) (Ref).

Otitis media, acute

Otitis media, acute (alternative agent for mild [nonanaphylactic] penicillin allergy) (off-label): Oral: 500 mg twice daily; duration of therapy is 5 to 7 days (mild to moderate infection) **or** 10 days (severe infection) (Ref).

Pneumonia, community acquired, outpatient empiric therapy

Pneumonia, community acquired, outpatient empiric therapy (patients with comorbidities): Oral: 500 mg twice daily as part of an appropriate combination regimen. Duration is for a minimum of 5 days; patients should be clinically stable with normal vital signs before discontinuing therapy (Ref).

Streptococcal pharyngitis, group A

Streptococcal pharyngitis, group A (alternative agent for mild, nonanaphylactic penicillin allergy):

Note: Cephalosporin selection depends on the type of hypersensitivity reaction to penicillin. To avoid the development of resistance, narrower spectrum cephalosporins (eg, cephalexin or cefadroxil) are preferred when possible (Ref).

Oral: 250 mg twice daily for 10 days (Ref).

Surgical prophylaxis

Surgical prophylaxis (eg, cardiac surgery, head and neck surgery) (alternative agent): IV: 1.5 g within 60 minutes prior to surgical incision; use in combination with metronidazole for select head and neck procedures. Cefuroxime dose may be repeated intraoperatively in 4 hours if procedure is lengthy or if there is excessive blood loss (Ref). In cases where an extension of prophylaxis is warranted postoperatively, total duration should be ≤24 hours (Ref). Postoperative prophylaxis is not recommended in clean and clean-contaminated surgeries (Ref).

Urinary tract infection

Urinary tract infection (alternative agent):

Note: Use only when first-line agents cannot be used; limited evidence suggests inferior efficacy of oral beta-lactams (Ref).

Cystitis, acute uncomplicated or acute simple cystitis (infection limited to the bladder without signs/symptoms of upper tract, prostate, or systemic infection): **Oral:** 250 mg twice daily for 5 to 7 days (Ref).

Urinary tract infection, complicated (including pyelonephritis) (off-label use): **Oral:** 500 mg twice daily (Ref); for patients with symptomatic improvement within the first ~48 hours of therapy, total duration is 5 to 7 days (7 days if therapy is completed with cefuroxime) (Ref).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Adult

The renal dosing recommendations are based upon the best available evidence and clinical expertise. Senior Editorial Team: Bruce Mueller, PharmD, FCCP, FASN, FNKF; Jason A. Roberts, PhD, BPharm (Hons), B App Sc, FSHP, FISAC; Michael Heung, MD, MS.

Altered kidney function:

Cefuroxime Dose Adjustments in Altered Kidney Function

CrCl (mL/minute)

IV

Oral

If usual recommended dose is 750 mg to 1.5 g every 8 hours

If usual recommended dose is 250 to 500 mg every 12 hours

[a]Expert opinion derived from concentrations observed in Konishi 1993.

≥30

No dosage adjustment necessary

No dosage adjustment necessary

10 to 30

750 mg to 1.5 g every 12 hours

250 mg every 12 hours (preferred)[a] or 250 to 500 mg every 24 hours

<10

750 mg to 1.5 g every 24 hours

250 mg every 24 hours (preferred)[a] or 250 to 500 mg every 48 hours

Augmented renal clearance (measured urinary CrCl ≥130 mL/minute/1.73 m^2): Augmented renal clearance (ARC) is a condition that occurs in certain critically ill patients without organ dysfunction and with normal serum creatinine concentrations. Young patients (<55 years of age) admitted post-trauma or major surgery are at highest risk for ARC, as well as those with sepsis, burns, or hematologic malignancies. An 8- to 24-hour measured urinary CrCl is necessary to identify these patients (Ref).

IV:

Extended infusion: 1.5 g infused over 3 hours every 6 hours (Ref).

Continuous infusion: 1.5 g loading dose, followed by 6 g infused over 24 hours every 24 hours (Ref).

Hemodialysis, intermittent (thrice weekly): Dialyzable (enhances plasma clearance by at least 30% (Ref)):

IV, Oral: Dose as for patients with CrCl <10 mL/minute; when scheduled dose falls on a dialysis day, administer after dialysis (Ref).

Peritoneal dialysis:

IV, Oral: Dose as for patients with CrCl <10 mL/minute (Ref).

CRRT: Drug clearance is dependent on the effluent flow rate, filter type, and method of renal replacement. Recommendations are based on high-flux dialyzers and effluent flow rates of 20 to 25 mL/kg/hour (or ~1,500 to 3,000 mL/hour) unless otherwise noted. Appropriate dosing requires consideration of adequate drug concentrations (eg, site of infection) and consideration of initial loading doses. Close monitoring of response and adverse reactions (eg, neurotoxicity) due to drug accumulation is important.

IV: 1.5 g every 12 hours (Ref).

PIRRT (eg, sustained, low-efficiency diafiltration): Drug clearance is dependent on the effluent flow rate, filter type, and method of renal replacement. Appropriate dosing requires consideration of adequate drug concentrations (eg, site of infection) and consideration of initial loading doses. Close monitoring of response and adverse reactions (eg, neurotoxicity) due to drug accumulation is important.

IV: 1.5 g every 12 hours (Ref).

Dosing: Liver Impairment: Adult

The liver dosing recommendations are based upon the best available evidence and clinical expertise. Senior Editorial Team: Matt Harris, PharmD, MHS, BCPS, FAST, FCCP; Jeong Park, PharmD, MS, BCXTP, FCCP, FAST; Arun Jesudian, MD; Sasan Sakiani, MD.

Liver impairment prior to treatment initiation:

Child-Turcotte-Pugh class A to C: No dosage adjustment necessary (Ref).

Dosing: Older Adult

Refer to adult dosing.

Dosing: Pediatric

(For additional information see ”Cefuroxime: Pediatric drug information”)

General dosing:

Infants, Children, and Adolescents:

IV, IM: 100 to 150 mg/kg/day in divided doses every 8 hours; maximum daily dose: 6,000 mg/**day** (Ref).

Oral: 20 to 30 mg/kg/day in divided doses every 12 hours; maximum daily dose: 1,000 mg/**day**; higher doses may be used for some indications (Ref).

Chronic bronchitis, acute bacterial exacerbation

Chronic bronchitis, acute bacterial exacerbation: Adolescents: Oral: 250 to 500 mg every 12 hours for 10 days.

Intra-abdominal infection

Intra-abdominal infection (alternative agent): Limited data available:

Infants, Children, and Adolescents: IV: 150 to 200 mg/kg/day in divided doses every 6 to 8 hours as part of an appropriate combination regimen; maximum dose: 1,500 mg/dose. Treatment duration varies based on specific source of infection, success of source control procedures, and clinical response; a total duration of 5 to 7 days following source control is typically recommended (Ref).

Lyme disease

Lyme disease (*Borrelia* spp. infection): Infants, Children, and Adolescents: Oral: 30 mg/kg/day in divided doses every 12 hours; maximum dose: 500 mg/dose. Duration of therapy depends on clinical syndrome; treat erythema migrans and borrelial lymphocytoma for 14 days, carditis for 14 to 21 days, arthritis (initial, recurrent, or refractory) for 28 days, and acrodermatitis chronica atrophicans for 21 to 28 days (Ref).

Osteoarticular infection, acute

Osteoarticular infection, acute (eg, septic [bacterial] arthritis, osteomyelitis):

IV, IM: Infants ≥3 months, Children, and Adolescents: IV, IM: 150 to 200 mg/kg/day in divided doses every 6 to 8 hours; maximum daily dose: 6,000 mg/**day** (Ref).

Oral: Limited data available: Infants ≥3 months, Children, and Adolescents: Oral: 60 to 100 mg/kg/day in divided doses every 8 hours; maximum dose: 1,000 mg/dose (Ref).

Duration of therapy: Minimum total duration (IV plus oral therapy) is 2 to 3 weeks for septic arthritis and 3 to 4 weeks for osteomyelitis; however, duration should be individualized based on several factors including causative pathogen, response to therapy, and normalization of inflammatory markers (Ref).

Otitis media, acute

Otitis media, acute (AOM) (alternative agent for nonanaphylactic penicillin allergy):

Infants ≥3 months, Children, and Adolescents: Oral: 30 mg/kg/day in divided doses every 12 hours; maximum dose: 500 mg/dose (Ref). For patients with severe or recurrent AOM, tympanic membrane perforation, or who are <2 years of age, treat for 10 days; for patients ≥2 years of age with mild to moderate, nonrecurrent disease without tympanic membrane perforation, shorter durations of 5 to 7 days may be sufficient (Ref).

Pneumonia, bacterial

Pneumonia, bacterial (alternative agent): Infants and Children: HIV-exposed/-infected: IV: 105 to 150 mg/kg/day in divided doses every 8 hours; maximum dose: 2,000 mg/dose (Ref).

Rhinosinusitis, acute bacterial

Rhinosinusitis, acute bacterial: Infants ≥3 months, Children, and Adolescents: Oral: 30 mg/kg/day in divided doses every 12 hours for 10 days; maximum dose: 500 mg/dose. Manufacturer labeling suggests that children and adolescents may also receive 250 mg in oral tablets every 12 hours (Ref). Limited data exist to determine optimal duration; historically 10 days has been recommended, but recently some have suggested a duration as short as 5 days; some patients may require extended therapy (ie, 14 to 21 days) based on clinical resolution (Ref).

Skin and soft tissue infection

Skin and soft tissue infection (SSTI) (including impetigo): Note: Alternative agents are preferred for first-line treatment of SSTI (eg, first-generation cephalosporins) (Ref).

Oral: Infants ≥3 months, Children, and Adolescents: Oral: 30 mg/kg/day in divided doses every 12 hours; maximum dose: 500 mg/dose (Ref). For impetigo, treat for 7 days; typical duration for other uncomplicated skin and soft tissue infection is 5 days, but therapy may be extended if clinical response is inadequate (Ref).

IV: Infants ≥3 months and Children ≤11 years: IV: 50 to 100 mg/kg/day in divided doses every 6 to 8 hours; maximum dose: 1,500 mg/dose. Transition to oral therapy when able; total duration dependent on presentation, severity of infection, and clinical response (Ref).

Streptococcus, group A; pharyngitis/tonsillitis

Streptococcus, group A; pharyngitis/tonsillitis (alternative agent for nonanaphylactic penicillin allergy): Note: Narrow-spectrum cephalosporins (eg, cephalexin) are preferred over broad-spectrum cephalosporins such as cefuroxime (Ref).

Infants ≥3 months, Children, and Adolescents: Oral: 20 mg/kg/day in divided doses every 12 hours for 10 days; maximum dose: 250 mg/dose (Ref).

Surgical prophylaxis

Surgical prophylaxis: Children and Adolescents: IV: 50 mg/kg within 60 minutes prior to incision; may repeat dose in 4 hours if procedure is lengthy or if there is excessive blood loss; maximum dose: 1,500 mg/dose (Ref).

Urinary tract infection

Urinary tract infection: Infants, Children, and Adolescents: Limited data available in ages ≤12 years: Oral: 20 to 30 mg/kg/day in divided doses every 12 hours; maximum daily dose: 1,000 mg/day. European guidelines suggest that this dose may be administered in divided doses every 8 hours. Manufacturer labeling suggests that adolescents with uncomplicated infection may receive 250 mg every 12 hours (Ref). Duration of therapy should be individualized based on patient age, severity/extent of infection, and clinical response; typical duration is 7 to 14 days, though it may be as short as 3 to 5 days (eg, for uncomplicated cystitis in patients ≥2 years of age) (Ref).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Pediatric

Altered kidney function:

Infants, Children, and Adolescents: **Note:** For oral therapy, there are no dosage adjustments provided in the manufacturer's labeling; for IV therapy, the manufacturer recommends similar adjustments to those provided for adults. However, the following guidance has been used by some clinicians (Ref).

Cefuroxime Dosage Adjustments in Altered Kidney Function

GFR

IV

Oral

If usual recommended dose is 75 to 150 mg/kg/**day** divided every 8 hours

If usual recommended dose is 30 mg/kg/**day** divided every 12 hours

Usual dose: 25 to 50 mg/kg/dose every 8 hours

Usual dose: 15 mg/kg/dose every 12 hours

≥30 mL/minute/1.73 m^2

No dosage adjustment necessary

No dosage adjustment necessary

10 to 29 mL/minute/1.73 m^2

25 to 50 mg/kg/dose every 12 hours

No dosage adjustment necessary

<10 mL/minute/1.73 m^2

25 to 50 mg/kg/dose every 24 hours

15 mg/kg/dose every 24 hours

Intermittent hemodialysis

25 to 50 mg/kg/dose every 24 hours; administer after dialysis on dialysis days

15 mg/kg/dose every 24 hours; administer after dialysis on dialysis days

Peritoneal dialysis (PD)

25 to 50 mg/kg/dose every 24 hours

15 mg/kg/dose every 24 hours

CRRT:

Infants, Children, and Adolescents:

Note: Drug clearance is dependent on the effluent flow rate, filter type, and method of renal replacement. Appropriate dosing requires consideration of drug penetration to site of infection, minimal inhibitory concentration (MIC) of bacteria, severity of illness, receipt of any initial loading doses, etc. Due to minimal data, consider monitoring serum concentrations if available.

IV: 25 to 50 mg/kg/**dose** every 8 hours (Ref); intervals of every 12 to 18 hours have also been suggested depending on effluent flow rate (Ref).

Dosing: Liver Impairment: Pediatric

There are no dosage adjustments provided in the manufacturer's labeling.

Adverse Reactions

The following adverse drug reactions and incidences are derived from product labeling unless otherwise specified.

>10%: Gastrointestinal: Diarrhea (4% to 11%)

1% to 10%:

Endocrine & metabolic: Increased lactate dehydrogenase (1%)

Gastrointestinal: Nausea and vomiting (3% to 7%)

Genitourinary: Vaginitis (≤5%)

Hematologic & oncologic: Decreased hematocrit (≤10%), decreased hemoglobin (≤10%), eosinophilia (1% to 7%)

Hepatic: Increased serum alanine aminotransferase (2%), increased serum alkaline phosphatase (2%), increased serum aspartate aminotransferase (2%)

Immunologic: Jarisch-Herxheimer reaction (6%)

Local: Local thrombophlebitis (2%)

<1%:

Cardiovascular: Chest pain, chest tightness, tachycardia

Dermatologic: Erythema of skin, pruritus, skin rash, urticaria

Endocrine & metabolic: Increased thirst

Gastrointestinal: Abdominal cramps, abdominal pain, anorexia, dyspepsia, flatulence, oral mucosa ulcer

Genitourinary: Dysuria, urethral bleeding, urethral pain, vaginal discharge, vulvovaginal candidiasis, vulvovaginal pruritus

Hematologic & oncologic: Neutropenia, positive direct Coombs test

Hepatic: Hyperbilirubinemia

Hypersensitivity: Swollen tongue

Nervous system: Chills, dizziness, drowsiness, headache, trismus

Neuromuscular & skeletal: Muscle cramps, muscle rigidity, muscle spasm (neck)

Otic: Hearing loss

Renal: Renal pain

Respiratory: Dyspnea

Miscellaneous: Drug fever

Postmarketing:

Dermatologic: Erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis

Gastrointestinal: Cholestasis, *Clostridioides difficile*–associated diarrhea, *Clostridioides difficile* colitis, colitis

Hematologic & oncologic: Hemolytic anemia, leukopenia, pancytopenia, prolonged prothrombin time, thrombocytopenia

Hepatic: Hepatitis, jaundice

Hypersensitivity: Anaphylaxis, angioedema, hypersensitivity angiitis, hypersensitivity reaction (may include ischemic heart disease with or without acute myocardial infarction), serum sickness–like reaction

Nervous system: Encephalopathy, seizure

Renal: Decreased creatinine clearance, increased blood urea nitrogen, increased serum creatinine, interstitial nephritis, kidney impairment

Contraindications

Hypersensitivity to cefuroxime, any component of the formulation, or other beta-lactam antibacterial drugs (eg, penicillins and cephalosporins)

Warnings/Precautions

Concerns related to adverse effects:

• Elevated INR: May be associated with increased INR, especially in nutritionally-deficient patients, prolonged treatment, hepatic or renal disease.

• Hypersensitivity reactions: Serious and occasionally severe or fatal hypersensitivity (anaphylactic) reactions have been reported in patients receiving beta-lactam drugs. Before initiating therapy, carefully investigate previous penicillin, cephalosporin, or other allergen hypersensitivity. Use caution if given to a patient with a penicillin or other beta-lactam allergy because cross sensitivity among beta-lactam antibacterial drugs has been established. If an allergic reaction occurs, discontinue and institute appropriate therapy.

• Superinfection: Prolonged use may result in fungal or bacterial superinfection, including *C. difficile*-associated diarrhea (CDAD) and pseudomembranous colitis; CDAD has been observed >2 months postantibiotic treatment.

Disease-related concerns:

• Gastrointestinal disease: Use with caution in patients with a history of colitis.

• Renal impairment: Use with caution in patients with renal impairment; modify dosage in severe impairment.

• Seizure disorders: Use with caution in patients with a history of seizure disorder; cephalosporins have been associated with seizure activity, particularly in patients with renal impairment not receiving dose adjustments. Discontinue if seizures occur.

Dosage form specific issues:

• Benzyl alcohol and derivatives: Some dosage forms may contain sodium benzoate/benzoic acid; benzoic acid (benzoate) is a metabolite of benzyl alcohol; large amounts of benzyl alcohol (≥99 mg/kg/day) have been associated with a potentially fatal toxicity (“gasping syndrome”) in neonates; the “gasping syndrome” consists of metabolic acidosis, respiratory distress, gasping respirations, CNS dysfunction (including convulsions, intracranial hemorrhage), hypotension, and cardiovascular collapse (AAP [”Inactive” 1997]; CDC 1982); some data suggests that benzoate displaces bilirubin from protein binding sites (Ahlfors 2001); avoid or use dosage forms containing benzyl alcohol derivative with caution in neonates. See manufacturer’s labeling.

• Phenylalanine: Some products may contain phenylalanine.

• Tablets: Should not be crushed or chewed due to a strong, persistent bitter taste.

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution Reconstituted, Injection, as sodium [strength expressed as base, preservative free]:

Generic: 750 mg (1 ea)

Solution Reconstituted, Intravenous, as sodium [strength expressed as base, preservative free]:

Generic: 1.5 g (1 ea)

Tablet, Oral, as axetil [strength expressed as base]:

Generic: 250 mg, 500 mg

Generic Equivalent Available: US

Yes

Pricing: US

Solution (reconstituted) (Cefuroxime Sodium Injection)

750 mg (per each): $2.70 - $3.67

Solution (reconstituted) (Cefuroxime Sodium Intravenous)

1.5 g (per each): $5.40 - $7.01

Tablets (Cefuroxime Axetil Oral)

250 mg (per each): $4.40 - $5.66

500 mg (per each): $5.50 - $11.11

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer’s AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Dosage Forms: Canada

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution Reconstituted, Intravenous, as sodium [strength expressed as base]:

Generic: 750 mg (1 ea); 1.5 g (1 ea); 7.5 g (1 ea)

Suspension Reconstituted, Oral, as axetil [strength expressed as base]:

Ceftin: 125 mg/5 mL (70 mL, 100 mL) [contains aspartame]

Tablet, Oral, as axetil [strength expressed as base]:

Generic: 250 mg, 500 mg

Extemporaneous Preparations

10 mg/mL Oral Liquid

Note: Use dedicated or disposable equipment, follow appropriate cleaning procedures, and avoid dust (ie, by wetting with vehicle and mixing carefully) to avoid contamination of compounding area since product is a beta-lactam (Ref).

A 10 mg/mL oral liquid may be made with cefuroxime axetil tablets and simple syrup. To prepare 100 mL, calculate the number of tablets needed to equal cefuroxime 1,000 mg; crush required number of tablets in a mortar and reduce to a fine powder. Pour a small quantity of simple syrup into the mortar and mix to form a smooth paste; add additional simple syrup in incremental amounts to final volume of 100 mL, mixing thoroughly after each addition. Package in a tight, light-resistant container and label ”shake well” and ”refrigerate.” May be stored for up to 28 days refrigerated (Ref).

Administration: Adult

Oral suspension [Canadian product]: Administer with food. Shake well before use.

Oral tablet: May administer with or without food (absorption improved with food). Swallow tablet whole (crushed tablet has strong, persistent, bitter taste).

IM: Inject deep IM into large muscle mass.

IV: Inject direct IV over 3 to 5 minutes. Infuse intermittent infusion over 15 to 30 minutes.

Preparation for Administration: Adult

Oral suspension [Canadian product]: Refer to manufacturer’s product labeling for reconstitution instructions (Ref).

Duplex container: Unlatch side tab, unfold, and remove foil strip from drug chamber. Point set port in downward direction, fold container just below the diluent meniscus, and squeeze the diluent chamber until the seal between the diluent and drug powder opens. Shake until dissolved.

Administration: Pediatric

Oral: Cefuroxime axetil suspension must be administered with food; shake suspension well before use; the dose may be added to cold milk, fruit juice, or lemonade prior to administration if desired (Canadian product). Tablets may be administered with or without food; administer with food to increase absorption; avoid crushing the tablet due to its bitter taste.

Parenteral:

IM: Inject deep IM into large muscle mass, such as gluteus or lateral part of thigh.

Intermittent IV infusion: Administer over 15 to 30 minutes.

IV push: Administer over 3 to 5 minutes.

Preparation for Administration: Pediatric

Oral suspension: Reconstitute powder for oral suspension with appropriate amount of water as specified on the bottle. Shake vigorously until suspended.

Parenteral:

IM: Dilute 750 mg vial with 3 mL SWFI; resultant concentration: ~225 mg/mL.

IV: Dilute 750 mg vial with 8.3 mL SWFI or 1,500 mg vial with 16 mL SWFI to a final concentration of ~90 mg/mL (per manufacturer); some centers have used a final concentration of 100 mg/mL.

Intermittent IV infusion: Further dilute reconstituted solution to a final concentration ≤30 mg/mL; in fluid-restricted patients, dilution with SWFI to a concentration of 137 mg/mL results in a maximum recommended osmolality for peripheral infusion (Ref).

Storage/Stability

Injection: Store intact vials at 15°C to 30°C (59°F to 86°F); protect from light. Reconstituted solution is stable for 24 hours at room temperature and 48 hours when refrigerated. IV infusion in NS or D5W solution is stable for 24 hours at room temperature, 7 days when refrigerated, or 26 weeks when frozen. After freezing, thawed solution is stable for 24 hours at room temperature or 21 days when refrigerated.

Duplex container: Store unactivated units at 20°C to 25°C (68°F to 77°F). Unactivated units with foil strip removed from the drug chamber must be protected from light and used within 7 days. Once activated, may be stored for up to 24 hours at room temperature or for 7 days under refrigeration. Do not freeze.

TwistVial vials: Joined, but not activated, vials are stable for 14 days. Once activated, stable for 24 hours at room temperature and 7 days refrigerated. Do not freeze.

Premix Galaxy plastic containers: Store frozen at -20°C. Thaw container at room temperature or under refrigeration; do not force thaw. Thawed solution is stable for 24 hours at room temperature and 28 days refrigerated; do not refreeze.

Oral suspension [Canadian product]: Prior to reconstitution, store at 2°C to 30°C (36°F to 86°F). Reconstituted suspension is stable for 10 days at 2°C to 8°C (36°F to 46°F).

Tablet: Store at 15°C to 30°C (59°F to 86°F).

Use: Labeled Indications

Bone and joint infections (injection only): Treatment of bone and joint infections caused by *Staphylococcus aureus* (penicillinase- and non-penicillinase-producing strains).

Chronic obstructive pulmonary disease, acute exacerbation (tablets only): Treatment of mild to moderate acute bacterial exacerbations of chronic bronchitis in adults and adolescents ≥13 years of age caused by *Streptococcus pneumoniae, Haemophilus influenzae* (beta-lactamase negative strains), or *Haemophilus parainfluenzae* (beta-lactamase negative strains).

Lower respiratory tract infections (injection only): Treatment of lower respiratory tract infections, including pneumonia, caused by *S. pneumoniae*, *H. influenzae* (including ampicillin-resistant strains), *Klebsiella* spp., *S. aureus* (penicillinase- and non-penicillinase-producing strains), *Streptococcus pyogenes,* and *Escherichia coli*.

Lyme disease (early) (tablets only): Treatment of adults and adolescents ≥13 years of age with early Lyme disease (eg, erythema migrans, carditis) caused by *Borrelia burgdorferi*.

Otitis media, acute (tablets and oral suspension [Canadian product] only): Treatment of pediatric patients ≥3 months of age with acute bacterial otitis media caused by *S. pneumoniae*, *H. influenzae* (including beta-lactamase-producing strains), *Moraxella catarrhalis* (including beta-lactamase-producing strains), or *S. pyogenes.*

Septicemia (injection only): Treatment of septicemia caused by *S. aureus* (penicillinase- and non-penicillinase-producing strains), *S. pneumoniae*, *E. coli*, *H. influenzae* (including ampicillin-resistant strains), and *Klebsiella* spp.

Sinusitis, acute bacterial (tablets only): Treatment of mild to moderate acute bacterial maxillary sinusitis caused by *S. pneumoniae* or *H. influenzae* (non-beta-lactamase-producing strains only).

Limitations of use: Effectiveness for sinus infections caused by beta-lactamase-producing *H. influenzae* or *M. catarrhalis* in patients with acute bacterial maxillary sinusitis has not been established. **Note**: According to the IDSA guidelines for acute bacterial rhinosinusitis, cefuroxime is no longer recommended as monotherapy for initial empiric treatment (IDSA [Chow 2012]).

Skin and skin-structure infections (injection; tablets [uncomplicated infections only]; oral suspension [Canadian product]): Treatment of adults and pediatric patients >3 months of age with skin and skin-structure infections (including impetigo) caused by *S. aureus* (penicillinase- and non-penicillinase-producing strains), *S. pyogenes*, *E. coli*, *Klebsiella* spp., and *Enterobacter* spp.

Streptococcal pharyngitis, group A (tablets and oral suspension [Canadian product] only): Treatment of mild to moderate pharyngitis/tonsillitis caused by *S. pyogenes* in adults and pediatric patients ≥3 months of age.

Limitations of use: Efficacy in the prevention of rheumatic fever has not been established in clinical trials. Efficacy in the treatment of penicillin-resistant strains of *S. pyogenes* has not been demonstrated.

Surgical prophylaxis (injection only): Prophylaxis of infection in patients undergoing surgical procedures that are classified as clean-contaminated or potentially contaminated procedures.

Urinary tract infection, uncomplicated (tablets and injection only) or complicated (injection only): Treatment of adults and pediatric patients >3 months of age with urinary tract infection caused by *E. coli* and *Klebsiella* spp.

Use: Off-Label: Adult

Bite wound, prophylaxis or treatment (animal or human bite); Intra-abdominal infection, mild to moderate, community acquired in patients without risk factors for resistance or treatment failure; Lyme disease (Borrelia spp. infection), arthritis without neurologic involvement; Odontogenic soft tissue infection, pyogenic; Urinary tract infection, complicated (including pyelonephritis)

Medication Safety Issues

Sound-alike/look-alike issues:

Cefuroxime may be confused with cefotaxime, cefprozil, deferoxamine, sulfaSALAzine

Ceftin may be confused with Cefzil, Cipro

Older Adult: High-Risk Medication:

Cefuroxime is identified in the Screening Tool of Older Person's Prescriptions (STOPP) criteria as a potentially inappropriate medication in older adults (≥65 years of age) for use in asymptomatic bacteriuria (O'Mahony 2023).

International issues:

Ceftin [US, Canada] may be confused with Cefiton brand name for cefixime [Portugal]; Ceftim brand name for ceftazidime [Portugal]; Ceftime brand name for ceftazidime [Thailand]

Metabolism/Transport Effects

Substrate of OAT1/3

Drug Interactions

(For additional information: Launch drug interactions program)

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within "CYP3A4 Inducers [Strong]" are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the "Launch drug interactions program" link above.

Aescin: Antibiotics may increase serum concentrations of Aescin. Management: Consider alternatives to this combination as aescin product labeling states coadministration with antibiotics is not recommended. *Risk D: Consider Therapy Modification*

Aminoglycosides: Cephalosporins may increase nephrotoxic effects of Aminoglycosides. Cephalosporins may decrease serum concentrations of Aminoglycosides. *Risk C: Monitor*

Antacids: May decrease serum concentrations of Cefuroxime. Management: Administer cefuroxime axetil at least 1 hour before or 2 hours after the administration of short-acting antacids. Taking cefuroxime with food may lessen the magnitude of this interaction. *Risk D: Consider Therapy Modification*

Bacillus Calmette Guerin (BCG) (Intravesical): Antibiotics may decrease therapeutic effects of Bacillus Calmette Guerin (BCG) (Intravesical). *Risk X: Avoid*

Bacillus Calmette Guerin (BCG) (Percutaneous): Antibiotics may decrease therapeutic effects of Bacillus Calmette Guerin (BCG) (Percutaneous). *Risk C: Monitor*

Bacillus clausii: Antibiotics may decrease therapeutic effects of Bacillus clausii. Management: Bacillus clausii should be taken in between antibiotic doses during concomitant therapy. *Risk D: Consider Therapy Modification*

Cholera Vaccine: Antibiotics may decrease therapeutic effects of Cholera Vaccine. Management: Avoid cholera vaccine in patients receiving systemic antibiotics, and within 14 days following the use of oral or parenteral antibiotics. *Risk X: Avoid*

Fecal Microbiota (Live) (Oral): Antibiotics may decrease therapeutic effects of Fecal Microbiota (Live) (Oral). *Risk X: Avoid*

Fecal Microbiota (Live) (Rectal): Antibiotics may decrease therapeutic effects of Fecal Microbiota (Live) (Rectal). *Risk X: Avoid*

Histamine H2 Receptor Antagonists: May decrease absorption of Cefuroxime. Management: Avoid concomitant use of oral cefuroxime axetil and H2 receptor antagonists if possible. If combined, ensure oral cefuroxime axetil is taken with food to minimize the magnitude of this interaction. *Risk D: Consider Therapy Modification*

Immune Checkpoint Inhibitors (Anti-PD-1, -PD-L1, and -CTLA4 Therapies): Antibiotics may decrease therapeutic effects of Immune Checkpoint Inhibitors (Anti-PD-1, -PD-L1, and -CTLA4 Therapies). *Risk C: Monitor*

Inhibitors of the Proton Pump (PPIs and PCABs): May decrease absorption of Cefuroxime. Management: Avoid concomitant use of oral cefuroxime axetil and proton pump inhibitors (PPIs) or potassium-competitive acid blockers (PCABs) when possible. If combined, ensure oral cefuroxime axetil is taken with food to minimize the magnitude of this interaction. *Risk D: Consider Therapy Modification*

Lactobacillus and Estriol: Antibiotics may decrease therapeutic effects of Lactobacillus and Estriol. *Risk C: Monitor*

Mycophenolate: Antibiotics may decrease active metabolite exposure of Mycophenolate. Specifically, concentrations of mycophenolic acid (MPA) may be reduced. *Risk C: Monitor*

Probenecid: May increase serum concentrations of Cephalosporins. *Risk C: Monitor*

Sodium Picosulfate: Antibiotics may decrease therapeutic effects of Sodium Picosulfate. Management: Consider using an alternative product for bowel cleansing prior to a colonoscopy in patients who have recently used or are concurrently using an antibiotic. *Risk D: Consider Therapy Modification*

Typhoid Vaccine: Antibiotics may decrease therapeutic effects of Typhoid Vaccine. Only the live attenuated Ty21a strain is affected. Management: Avoid use of live attenuated typhoid vaccine (Ty21a) in patients being treated with systemic antibacterial agents. Postpone vaccination until 3 days after cessation of antibiotics and avoid starting antibiotics within 3 days of last vaccine dose. *Risk D: Consider Therapy Modification*

Vitamin K Antagonists: Cephalosporins may increase anticoagulant effects of Vitamin K Antagonists. *Risk C: Monitor*

Food Interactions

Bioavailability is increased with food; cefuroxime serum levels may be increased if taken with food or dairy products. Clinical and bacteriologic responses were independent of food intake in clinical trials.

Management: Administer tablet without regard to meals; suspension [Canadian product] must be administered with food.

Pregnancy Considerations

Cefuroxime crosses the placenta (Bousfield 1981; De Leeuw 1993; Holt 1994; Lalic-Popovic 2016; Roumen 1990).

An increased risk of major birth defects or other adverse fetal or maternal outcomes has generally not been observed following maternal use of cephalosporin antibiotics, including cefuroxime.

Due to pregnancy-induced physiologic changes, some pharmacokinetic properties of cefuroxime may be altered (Lalic-Popovic 2016; Philipson 1982).

Cefuroxime is one of the antibiotics effective for prophylactic use prior to cesarean delivery (ACOG 2018).

Cefuroxime is used for the treatment of Lyme disease. Vertical transmission from mother to fetus is not well documented; it is unclear if infection increases the risk of adverse pregnancy outcomes. When treatment for Lyme disease in pregnancy is needed, the indications and dosing of cefuroxime are the same as in nonpregnant patients (IDSA/AAN/ACR [Lantos 2021]; SOGC [Smith 2020]).

Breastfeeding Considerations

Cefuroxime is present in breast milk.

• Cefuroxime breast milk concentrations were evaluated in 2 lactating patients following a dose of 750 mg IV administered 3 times daily for 2 days following cesarean delivery. Breast milk was sampled on the second day 1 hour after the dose. Mean cefuroxime breast milk concentrations were 0.34 mcg/mL (patient 1) and 0.39 mcg/mL (patient 2). Mean maternal serum concentrations were 16.2 mcg/mL and 19 mcg/mL in patients 1 and 2, respectively (Kiriazopoulos 2019).

• Cefuroxime breast milk concentrations were evaluated in 2 lactating patients following an oral dose of 500 mg. Breast milk was sampled in intervals over 24 hours after the dose. Breast milk concentrations over 24 hours ranged from 0.0264 mcg/mL to 0.685 mcg/mL, with the peak cefuroxime concentrations occurring 2 hours after the dose (Kul 2023).

• Data are available from one patient treated with oral cefuroxime 500 mg 3 times a day for mastitis. Breast milk concentrations of cefuroxime 30 to 90 minutes after the dose ranged from 0.09 to 0.59 mcg/mL in the healthy breast and 0.57 to 1.05 mcg/mL in the infected breast (Nakamura 1987).

Diarrhea has been reported in breastfeeding infants exposed to cefuroxime (Benyami 2005). In general, antibiotics that are present in breast milk may cause non-dose-related modification of bowel flora. Monitor infants for GI disturbances, such as thrush and diarrhea (WHO 2002).

According to the manufacturer, the decision to breastfeed during therapy should consider the risk of infant exposure, the benefits of breastfeeding to the infant, and the benefits of treatment to the mother; however, cephalosporins are generally considered acceptable for use while breastfeeding (Ito 2000).

Dietary Considerations

Some products may contain phenylalanine and/or sodium.

Oral suspension [Canadian product]: Should be taken with food.

Monitoring Parameters

Monitor renal, hepatic, and hematologic function periodically with prolonged therapy. Monitor prothrombin time in patients at risk of prolongation during cephalosporin therapy (nutritionally-deficient, prolonged treatment, renal or hepatic disease). Observe for signs and symptoms of anaphylaxis during first dose.

Mechanism of Action

Inhibits bacterial cell wall synthesis by binding to one or more of the penicillin-binding proteins (PBPs) which in turn inhibits the final transpeptidation step of peptidoglycan synthesis in bacterial cell walls, thus inhibiting cell wall biosynthesis. Bacteria eventually lyse due to ongoing activity of cell wall autolytic enzymes (autolysins and murein hydrolases) while cell wall assembly is arrested.

Pharmacokinetics (Adult Data Unless Noted)

Absorption: Oral tablet: Increases with food.

Distribution: Widely to body tissues and fluids including bronchial secretions, synovial and pericardial fluid, kidneys, heart, liver, bone and bile; crosses blood-brain barrier.

V_d:

Infants and children ≤6 years of age: 0.65 L/kg (del Rio 1982).

Adults: 50 ± 28 L (Nix 1997).

Brain tissue:blood ratio, steady state (AUC): 33% (Hosman 2018).

Protein binding: 33% to 50%.

Metabolism: Cefuroxime axetil (oral) is hydrolyzed in the intestinal mucosa and blood to cefuroxime.

Bioavailability: Tablet: Fasting: 37%; Following food: 52%; Cefuroxime axetil suspension is less bioavailable than the tablet (91% of the AUC for tablets).

Half-life elimination:

Premature neonates:

PNA ≤3 days: Median: 5.8 hours (de Louvois 1982).

PNA ≥8 days: Median: 1.6 to 3.8 hours (de Louvois 1982).

Children and adolescents: 1.4 to 1.9 hours.

Adults: ~1 to 2 hours; prolonged with renal impairment.

Time to peak, serum: IM: ~15 to 60 minutes; IV: 2 to 3 minutes; Oral: Children: ~3 to 4 hours; Adults: ~2 to 3 hours.

Excretion: Urine (66% to 100% as unchanged drug).

Pharmacokinetics: Additional Considerations (Adult Data Unless Noted)

Anti-infective considerations:

Parameters associated with efficacy: Time dependent, associated with free time (*f*T) drug concentration > minimum inhibitory concentration (MIC). Goal: ≥30% to 40% *f*T > MIC (bacteriostatic), ≥60% to 70% (bactericidal) (Craig 1998; Nielsen 2011; Turnidge 1998).

Expected drug exposure in patients with normal renal function:

Infants and children ≤6 years of age: C_{max} (peak): IV, single dose:

50 mg/kg: 105 ± 36 mg/L (del Rio 1982).

Children 7 to 12 years of age: C_{max} (peak): Oral (tablet), single dose:

500 mg (fed): 4.3 to 5.7 mg/L (Ginsburg 1985).

500 mg (fasting): 3.8 to 3.9 mg/L (Ginsburg 1985).

Adults: C_{max} (peak):

IV: 1.5 g, single dose: 100 mg/L.

Oral (tablet), steady state:

250 mg (fed): 4.1 mg/L.

500 mg (fed): 7 mg/L.

500 mg (fasted): 4.9 mg/L.

Postantibiotic effect: H. influenzae: 0 to ~1.4 hours; *M. catarrhalis*: 0 to ~1.4 hours; *S. pneumoniae*: ~0.2 to 2.9 hours; *S. pyogenes*: 0 to ~1.75 hours; *S. aureus*: Not observed in most isolates (Craig 1993; Davies 2000; Dubois 2000).

Patient Counseling Points

(For additional information see ”Cefuroxime: Patient drug information”)

What is this drug used for?

• It is used to treat or prevent bacterial infections.

All drugs may cause side effects. However, many people have no side effects or only have minor side effects. Call your doctor or get medical help if any of these side effects or any other side effects bother you or do not go away:

All products:

• Diarrhea, upset stomach, or throwing up

Tablets and liquid (suspension):

• Bad taste in your mouth

Injection:

• Irritation where the shot is given

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

• Not able to pass urine or change in how much urine is passed

• Dark urine or yellow skin or eyes

• Fever, chills, or sore throat; any unexplained bruising or bleeding; or feeling very tired or weak

• Pale skin

• Seizures

• Vaginal itching or discharge

• Hearing loss

• C diff–associated diarrhea (CDAD) like stomach pain, cramps, or very loose, watery, or bloody stools

• Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

Consumer Information Use and Disclaimer: This information should not be used to decide whether or not to take this medicine or any other medicine. Only the healthcare provider has the knowledge and training to decide which medicines are right for a specific patient. This information does not endorse any medicine as safe, effective, or approved for treating any patient or health condition. This is only a limited summary of general information about the medicine’s uses from the patient education leaflet and is not intended to be comprehensive. This limited summary does NOT include all information available about the possible uses, directions, warnings, precautions, interactions, adverse effects, or risks that may apply to this medicine. This information is not intended to provide medical advice, diagnosis or treatment and does not replace information you receive from the healthcare provider. For a more detailed summary of information about the risks and benefits of using this medicine, please speak with your healthcare provider and review the entire patient education leaflet.

Brand Names: International

International Brand Names by Country

For country code abbreviations (show table)

- (AE) United Arab Emirates: Aprokam | Cefovex | Cefutil | Cefuzime | Ceroxim | Cetil | Daroxime | Maxil | Medo axetine | Neoxime | U cef | Zamur | Zenon | Zinacef | Zinnat | Zinoxime | Zinoximor;
- (AR) Argentina: Cefurox | Cefuroxima | Cefuroxima klonal | Cefuroxima norgreen | Cefuroxima richet | Fada cefuroxima;
- (AT) Austria: Aprokam | Cefuroxim | Cefuroxim aptapharma | Cefuroxim astro-pharma gmbh | Cefuroxim hexal pharma | Cefuroxim hikma | Cefuroxim kalceks | Cefuroxim MIP | Cefuroxim Sandoz | Cefuroxim sandoz | Curocef | Furoxim | Ximaract | Zinnat;
- (AU) Australia: Pharmacor cefuroxime | Zinnat;
- (BD) Bangladesh: Axefur | Axet | Axetil | Axibid | Axicef | Axim | Bactikil | Benkill | C 2 | Cefobac | Cefore | Cefotil | Cefroxil | Cefstar | Cefuact | Cefusil | Cefuson | Cefuxet | Ceroxime | Cexitil | Famicef | Fixcef | Furex | Furocef | Furotec | Furotil | Furotixol | Fuxtil | Kfore | Kilbac | Kilmax | Lepath | Libotil | Menat | Merocef | Mextil | Orextil | Picocef | Primocef | Probac | Recofast | Rofurox | Roxcef | Roxibac | Roxicil | Roxilab | Roximax | Roxinat | Secomax | Sefur | Sefurox | Sharpkil | Staxim | Til | Turbocef | Uroxime | Vexotil | Winfax | Xefrim | Ximetil | Ximorox | Xinarox | Xitil | Xorimax | Zerotil | Zinacef | Zinatil | Zinnat;
- (BE) Belgium: Aprokam | Cefurim | Cefuroxim fresenius kabi | Cefuroxim krka | Cefuroxim Mylan | Cefuroxim sodium sandoz | Cefuroxime bexal | Cefuroxime EG | Cefuroxime Sandoz | Cefuroxime sandoz | Doc cefuroxim | Doccefuro | Kefurox | Ximaract | Zinacef | Zinnat;
- (BF) Burkina Faso: Auroxetil | Cefrotil | Cfr 250 | Kefrox | Rocetil | Uroxime | Zamur | Zinnat;
- (BG) Bulgaria: Aksef | Axetine | Cefurocon | Cefuroxim | Cefuroxim abr | Cefuroxim MIP | Cefuroxim Panpharma | Cefuroxim tchaikapharma | Cefuroxime actavis | Furextil | Furocef | Kefurox | Lifurox | Neofuroxime | Ximaract | Xorimax | Zinacef | Zinnat;
- (BR) Brazil: Axetil cefuroxima | Axetilcefuroxima | Cefuroxima sodica | Keroxime | Medcef | Mefex | Totacef | Zencef | Zinacef | Zinnat;
- (CH) Switzerland: Aprokam | Cefurim eco | Cefuroxim actavis | Cefuroxim axapharm | Cefuroxim fresenius | Cefuroxim Labatec | Cefuroxim Mepha | Cefuroxim Sandoz | Cefuroxim spirig | Cefuroxim spirig hc | Cefuroxim Teva | Zinat;
- (CI) Côte d'Ivoire: Auroxetil | Cef2 | Ceforix | Cefuaxit | Cefuroxime sp | Cesavess | Cfr 250 | Furoxim | Lodifur | Melrox | Pulmocef | Rocetil | Sefurox | Xorimax | Zinnat | Zocef;
- (CL) Chile: Cefuroxima | Cerevax | Curocef | Favanex | Zinnat;
- (CN) China: An ke xin | Axetine | Ba xin | Cefuroxime | Da li xin | Fu le xin | Jia nuo xin | Kai di xin | Kai yi | Ke li di | Ku xin | Li fu xin | Li jian xin | Lifurox | Monacef | Pa wei xin | Pai wei xin | Pu li xin | Pu xin | Qing fu kang | Rui fu xin | Si pei ding | Supero | Xi lu xin | Xin fu xin | Xin lu xin | Ya xing | Yaxing | You le xin | Yun tai | Zinacef;
- (CO) Colombia: Binoxim | Cefome | Cefuroxima | Cefuroxima axetil | Cefxiro | Ocumisona | Xorimax | Zinacef | Zinnat;
- (CR) Costa Rica: Zamur | Zinnat;
- (CZ) Czech Republic: Aprokam | Axetine | Cefuroxim Kabi | Cefuroxim Pliva | Medoxin | Ricefan | Xorimax | Zinacef | Zinnat | Zinoxime | Znobact;
- (DE) Germany: Aprokam | Cefudura | Cefuhexal | Cefuro Puren | Cefurox | Cefurox basics | Cefurox basics sun | Cefuroxim | Cefuroxim actavis | Cefuroxim al | Cefuroxim alkem | Cefuroxim aristo | Cefuroxim ascend | Cefuroxim Aurus | Cefuroxim Axcount | Cefuroxim beta | Cefuroxim CT | Cefuroxim deltaselect | Cefuroxim dr. eberth | Cefuroxim Eberth | Cefuroxim fresenius | Cefuroxim heumann | Cefuroxim hexal | Cefuroxim hikma | Cefuroxim puren | Cefuroxim Ratiopharm | Cefuroxim saar | Cefuroxim sandoz | Cefuroxim Sandoz | Cefuroxim stada | Ceroxim | Elobact | Ximaract | Zinacef | Zinnat;
- (DO) Dominican Republic: Altacef | Cefuromac | Cefuroxima | Ceroxim | Neo erocetin | Zetnil |

Zinnat;

- (EC) Ecuador: Altacef | Axil | Betaxima | Cefulam | Cefur | Cefurex | Cefurmax | Cefurokron | Cefuroxima | Cefuroxint t | Cefusyn | Cefuzime | Celbix | Ceroxim | Excima | Furacam | Furoxicher | Furoxim | Italfur | Ozax | Toxedin | Xefurex | Ximax | Xorimax | Zamur | Zinnat;
- (EE) Estonia: Axetine | Cefoprim | Cefurox basics | Cefuroxim | Cefuroxime | Elobact | Furocef | Keroxime | Ketocef | Xorimax | Zinacef | Zinnat;
- (EG) Egypt: Cefumax | Ceroxim | Hebiuroxime | Oracef | Pancef | Zimocefox | Zinacef | Zinnat;
- (ES) Spain: Cefuroxima | Cefuroxima Apotex | Cefuroxima aurobindo | Cefuroxima aurovitas | Cefuroxima Bexal | Cefuroxima chiesi | Cefuroxima Cinfa | Cefuroxima combino | Cefuroxima Davur | Cefuroxima ges | Cefuroxima Ips | Cefuroxima Kern Pharma | Cefuroxima krka | Cefuroxima liderfarm | Cefuroxima mundogen | Cefuroxima normon | Cefuroxima Pensa | Cefuroxima Qualigen | Cefuroxima Ranbaxy | Cefuroxima Ratiopharm | Cefuroxima Reig Jofre | Cefuroxima Sala | Cefuroxima sandoz | Cefuroxima Stada | Cefuroxima stada genericos | Cefuroxima tecnigen | Cefuroxima teva | Curoxima | Lifurox | Nivador | Prokam | Selan | Ximaract | Zanetin | Zinnat;
- (ET) Ethiopia: Auroxetil | Cefuroxime | Cefuroxime synto | Gulf cefuroxime | Sanfur;
- (FI) Finland: Aprokam | Cefuroxim b. braun | Cefuroxim generics | Cefuroxim MIP pharma | Cefuroxim sandoz | Cefuroxim stragen | Cefuroxime Aurobindo | Cefuroxime orion | Kefurion | Zinacef | Zinnat;
- (FR) France: Aprokam | Cefuroxime Actavis | Cefuroxime Arrow | Cefuroxime bgr | Cefuroxime biogaran | Cefuroxime cristers | Cefuroxime Dci | Cefuroxime flavelab | Cefuroxime kabi | Cefuroxime krka | Cefuroxime merck | Cefuroxime Panpharma | Cefuroxime qualimed | Cefuroxime Ranbaxy | Cefuroxime Ratiopharm | Cefuroxime Sandoz | Cefuroxime Teva | Cefuroxime Winthrop | Cefuroxime zentiva | Cepazine | Iceca | Zinnat;
- (GB) United Kingdom: Aprokam | Britacef | Cefuroxime | Trav cefuroxime | Ximaract | Zinacef | Zinnat;
- (GH) Ghana: Enacef | Foxitil | Gebexime | Rocetil | Skycef;
- (GR) Greece: Anaptivan | Cefur | Cefuretil | Cefuroprol | Cefuroxime kabi | Ceruxim | Cupax | Feacef | Fredyr | Furaxil | Galemin | Gonif | Interbion | Medoxem | Mosalan | Nelabocin | Nipogalin | Normafenac | Normaphenac | Prokam | Receant | Saxetil | Vekfazolin | Vekfazoline | Yokel | Zetagal | Zilisten | Zinacef | Zinadol;
- (GT) Guatemala: Athera | Cefuroxima | Celza;
- (HK) Hong Kong: Apt Cefuroxime | Axacef | Axim | Ceflour | Cefuroxim stada | Cefuroxime shenzhen | Cefusan | Cmaxid | Farmacef | Furocef | Keroxime | Pulmocef | Quali Cefurnat | Sefuxim | Spizef | Teikeden | Tozef | Vick cefuroxime | Xorimax | Zenatop | Zinacef | Zinnat;
- (HR) Croatia: Axetine | Beloxim | Cefuroksim aptapharma | Cefuroksim JGL | Cefuroksim PharmaS | Cefuroxim MIP | Efox | Furocef | Novocef | Novuroxim | Xorimax | Zinnat;
- (HU) Hungary: Cefuroxim Kabi | Cefuroxim MIP | Cefuroxime aptapharma | Ceroxim | Furocef | Ximaract | Xorim | Zinacef | Zinnat;
- (ID) Indonesia: Anbacim | Cefurox | Cefuroxime | Celocid | Cethixim | Kalcef | Kenacef | Naroxit | Oxtercid | Roxbi | Sefure | Sharox | Situroxime | Soxime | Zinacef | Zinnat;
- (IE) Ireland: Aprok | Ceftal | Cefuroxime | Cefuroxime Actavis | Cefuroxime Teva | Zinacef | Zinnat;
- (IL) Israel: Cefurax | Ceroxim | Zinacef | Zinnat;
- (IN) India: Aaxim | Abactum | Abrocef | Actum | Acvaxim | Addicef | Adexim | Akucef | Altacef | Aquacef | Athxim o | Augxetil | Auroxetil | Axacef | Axeris | Axicef | Axifur | Axotil | Axtl | Bacticure | Bactilem | Barocef | Betaxime | Bifuroxy | Bigcef | Binex | Bipacef | Britum | C furo | C Rock | C rock 250 | C roxim | C tri t | C-tri | Cebox | Cef bet | Cef-Vepan | Cefac | Ceface | Cefactin | Cefadur ca | Cefakind | Cefam | Cefamax | Cefasyn | Cefax | Cefaxil | Cefbet | Cefblast | Cefbliss | Cefdove | Cefheal | Cefialfa | Cefking | Cefo | Cefogen | Cefompy | Cefona | Cefoprim | Cefoprit | Ceforange A | Ceforim | Cefoxim | Cefoxtil | Cefrotux | Cefrox | Cefsafe | Ceftalin | Ceftum | Cefuam | Cefuavenir | Cefucare | Cefuday | Cefudol | Cefuko | Cefulark | Cefulet | Cefuril | Cefurin | Cefurojet | Cefuronat | Cefuzact | Cefybond | Celfee | Cemax Cef | Cemax t | Cerom | Ceroxat | Ceroxim | Ceroxitum | Cetil | Ceurox | Cexum | Clasitil | Clobarium | Coraile | Covatil | Criticef h | CSF | Ct rox | Dericef | Dewflow | Dicef 500 | Dolocef | Ducati 500 | Duocin | Edrucef | Engel | Entocef | Evercef | Exeption | Exime | Fectiboon | Fervay | Flamicef | Folecef | Forcef | Foroxim | Foru | Foxer | Furix | Furobid | Furocef | Furodef | Furogen | Furogrey | Furomed | Furonix | Furox | Furoxivio | Garyfix | Genorox | Gericef | Greatum | Happycef | Highcef | Huroxime | Ibcef

| Idnaxim | Iviroxime | Jospar | Joxcy | Kefox | Kefstar | Keroxime | Konxime | Magxim | Maxim | Meftem | Megafuro | Metcef | Mic sef | Milcef | Mixie | Movacef | Mustcef | Neftum axt | Neroxim | Novacef | Novaroxim | Novotil | Ocef | Oltum | Omnixim | Oratil | Oxim | Prixicam | Pulmocef | Qtil | Quartum | Raxim | Refurox | Restum | Riocef | Rockcef | Royalcef | Ruxowel | Sayfur | Seazox | Secrox | Setum | Sifurox | Spectraxime | Spizef | Stacey | Stafcure | Supacef | Titil | Torfur | Travexel | Twicef | Urox | Viltacef | Warrior | Widecef | Wilcef | X til | Xe | Xeti | Xibid | Xim turbo | Ximbel | Ximoren | Xorimax | Xoxe | Zapisure | Zaxim | Zecil | Zefu | Zelfix | Zenoxim | Zimetile | Zitacef | Zocef | Zorbicef | Zovatil | Zuraxim | Zurocef | Zymocef | Zytum;

- (IQ) Iraq: Zinax;
- (IT) Italy: Aprokam | Biociclin | Cefamar | Cefoprim | Cefumax | Cefurin | Cefuroxima eg | Cefuroxima Mylan | Cefuroxima sandoz | Cefuroxima teva | Curoxim | Deltacef | Duxima | Itorex | Kefox | Lafurex | Monacef | Oraxim | Tilexim | Ximaract | Zinnat | Zinocep | Zoref;
- (JO) Jordan: Axetine | Cefovex | Ceftal | Cefutil | Cefuzime | Daroxime | Froxime | Gonif | Medo axetine | Nizacef | Oraxim | Supero | U cef | Xorimax | Zamur | Zilisten | Zinacef | Zinaxim | Zinnat | Zinoxime | Zinoximor;
- (JP) Japan: Oracef | Zinacef;
- (KE) Kenya: Altacef | Auroxetil | Axacef | Axetine | Bcef | C tri t | Cef2 | Cefact | Cefakind | Cefaks | Cefasyn | Cefaxil | Cefeal | Cefogen | Ceforce | Cefotil | Cefoxim | Cefrim | Cefuaxit | Cefuken | Cefuken ds | Cefuright | Cefurofast | Cefurome | Cefuroxime | Celza | Cerox | Cerox a | Cetil | Ceurox | Cortum | Curox | Dawacef | Evorox | Fetnal | Forcef | Foxitil | Furocef | Furomark | Furotil | Furox | Injroxime | Kefrox | Kefstar | Kefurox | Kilbac | Lebac | Mexirof | Mextil | Milcef | Mydawa cefuroxime | Nelcef | Proximexa | Pulmocef | Rofurox | Roxicef | Rufex | Sanfurox | Sayfur | Sefur | Sefur ds | Spizef | Syner cef | Theoroxime | Ximecor | Xorimax | Zamur | Zinacef | Zinavir ds | Zinaxime | Zinnat | Zocef | Zolidon;
- (KR) Korea, Republic of: Accef | Aju cefuroxime sodium | Alporin | Ausko cefuroxime sodium | Axcef | Bearcef | Binex cefuroxime sodium | Ceactil | Ceatil | Cefactil | Cefrox | Ceft | Ceftil | Cefuking | Cefur m | Cefurosaxime | Cefurotil | Cefuroxil | Cefuroxime | Cefuroxime sodium dae-wha | Cefuroxime sodium Huons | Cefuroxtil | Cefuroxtin | Cefusime | Cefutil | Cefutin | Cefuxetil | Cefuxil | Celcepoo | Cenet | Cepharoxim | Ceraf | Ceroxim | Cetroc | Cexil | Ckd cefuroxime sodium | Dongkoo cefuroxime sodium | Dongkwang cefuroxime axetil | Epufast | Furaxim | Fu-raxime | Furoxetil | Furoxim | Glocefuroxime | Gomcef | Hanlim cefuroxime sodium | Hupiroxime | Il yang cefuroxime sodium | Ildong cefuroxime | Jinnetil | Jw cefuroxime | Keroxim | Kyongbo cefuroxim axetil | L furoxime | Newgenpharm cefuroxime | Newroxime | Olcef | Samnam ce-furoxime | Samsung cefuroxime sodium | Samxetil | SCD Cefuroxime | Sencef | Seroxin | Shincef | Union cefuroxime sodium | White cefuroxim axetil | Wonfuroxime | Xincet | Zincef | Zincetil | Zinnat | Zinnetil;
- (KW) Kuwait: Aprokam | Cefovex | Cefutil | Cefuzime | Daroxime | Maxil | Xorim | Zamur | Zenon | Zinacef | Zinnat | Zinoximor;
- (LB) Lebanon: Axacef | Cefaks | Cefora | Cefovex | Cefumax | Cefurex | Cefuroxime | Cefutil | Cefuzime | Daroxime | Maxil | Xorim | Xorimax | Zenon | Zinacef | Zinnat | Zinoximor;
- (LT) Lithuania: Axetine | Biofuroksym | Cefakind | Cefogen | Cefurox basics | Cefuroxim dr. eberth | Cefuroxim fresenius kabi | Cefuroxima generis | Cefuroxime actavis | Cefuroxime ingen pharma | Cefuroxime MIP | Cefuroxin | Intefurox | Ketocef | Refoxim | Ricefan | Supero | Vekfazolin | Ximaract | Xorim | Xorimax | Zinacef | Zinnat;
- (LU) Luxembourg: Cefurim | Cefuroxim fresenius | Cefuroxim fresenius kabi | Cefuroxime EG | Zinacef | Zinnat;
- (LV) Latvia: Axetine | Cefasyn | Cefogen | Cefurax | Cefuroxime | Cefuroxime bcpp | Cefuroxime MIP | Elobact | Ketocef | Ricefan | Vekfazolin | Xorimax | Zinacef | Zinadol | Zinnat;
- (MA) Morocco: Cefutil | Ceroxim | Zinacef | Zinnat | Zinoxime;
- (MX) Mexico: Cefabiot | Cefagen | Cefantral | Cefuracet | Cefuroxima | Cefuroxima gi kend | Cefuroxima gi leme | Cefuroxima gi pisa | Cefuroxima gi prob | Cefuroxima Ivax | Cefuroxima Senosiain | Cefuroxima sodica | Cetoxil | Froxal | Fucerox | Furobioxin | Lemoxin | Lorbenil | Magnaspor | Nagaxy | Novador | Wayoxima | Ximaken | Zinnat;
- (MY) Malaysia: Anikef | Ceflour | Cefuroxime | Cefuroxime axetil zhijun | Ceroxime | Cetil | Cmaxid | Furoxime | Xorimax | Xylid | Zefu | Zilisten | Zinacef | Zinnat | Zocef;
- (NG) Nigeria: Acef | Akkaf cefuroxime axetil | Avrocef | Axacef | Bioessis | Bioxetil | Buroxim |

Ceezex | Cefanel xl | Cefsafe | Cefujet | Cefulight | Cefunat | Cefuraxe | Cefuromac | Cefuroxa | Cefuroxime | Ceroxim | Cetil | Chemocef | Clitex | Cuzi cefuroxime | Digicef | Doncinat | Dovxime | Exatil | Furoxetil | Gebexime | Intil | Kezitil | Lexibcure | Licafur | Lifeback cefuroxime | Malven | Miraxim | Olicef | Oxispa | Pemason cefuroxime | Pocco cefuroxime | Pontal cefuroxime | Proximexa | Pulmocef | Qceph | Quincef | Roxicef | Rufex | Sayfur | Sefzitil | Skp cefuroxime | Spizef | Talsure | Tryoxime ds | Upnat | Vaxcel cefuroxime | Xnat | Xymatyl | Zefef | Zinnat;

- (NL) Netherlands: Aprokam | Cefofix | Cefuroxim | Cefuroxim hikma | Cefuroximnatrium Sandoz | Ximaract | Zinacef | Zinnat;
- (NO) Norway: Aprokam | Cefuroxim | Cefuroxim 1a pharma | Cefuroxim b. braun | Cefuroxim navamedic | Cefuroxim Ratiopharm | Cefuroxim stada | Lifurox | Zinacef | Zinnat;
- (NZ) New Zealand: Cefuroxime | Cefuroxime actavis | Cefuroxime AFT | M cefuroxime | Zinacef | Zinnat;
- (PA) Panama: Cef 2 | Medoroxime | Zamur | Zinnat;
- (PE) Peru: Altacef | Cefaloxime | Cefstalip | Cefuroxima | Cefusim | Cefuxinil | Ceroxim | Fornax | Fucerox | Furabact | Furoxinol | Kilbac | Maxcefur | Oracef | Ozax | Priobron | Roxime | Supracef | Unotil | Ximagen | Zinnat | Zocef;
- (PH) Philippines: 2 gen | 2 gen scp | Acrocef | Aeroxime | Aeruginox | Alcefur | Alixime | Altacef | Altoxime | Ambifucef | Ambixime | Argixime | Axecrest | Axet | Axetine | Axurocef | Bermucef | Betcef | Biroxil | Cef u | Cefakind | Cefell med | Cefftabs | Cefiject | Cefnat | Cefogen | Cefopti | Cefosan | Cefovex | Cefrone | Cefrox | Cefstal | Ceft | Cefudyne | Cefueast | Cefufarm | Cefuget | Cefumax | Cefumex | Cefupen | Cefupharm | Cefuphil | Cefuplus | Cefura | Cefurax | Cefuremed | Cefurex | Cefurone | Cefurox | Cefuroxime | Cefusaph | Cefustar | Cefuvex | Cefuxcure | Cefuxime | Cefuzime | Cefwin | Cefzime | Cervin | Cesavess | Ceurox | Cevox | Cidokez | Cimex | Clovixime | Cmaxid | Cortum | Cosef | Curecef | Danoxime | Diacefarm | Educef | Efrox | Elixime | Eoroxime | Ephalox | Epirox | Eroxime | Eroxmit | Eurimax | Execore | Fetnal | Finax | Floxam | Fumerix | Furocef | Furocem | Furoxbet | Furoxy | Gavixime | Gencef | Goxime | Greenxime | Groxime | Groxone | Harox | Hiquacef | Ifurax | Infekor | Irefur | Jectocef | Kabiroxime | Kaftax | Kairoxi | Karixime | Keef 250 | Kefezy | Kefox | Kefstar | Kefsydin | Kefsyn | Kefurox | Kepox | Keroxim | Keunzef | Lasuzef | Laxime | Laxinat | Levexime | Lifurox | Medxil | Medxime | Medzyme | Mefurox | Microzef | Midrox | Mufexime | Myxetil | N xime | Neoxime | Nicocef | Panaxim | Pediacef | Philcef ds | Plerozef | Poweroxime | Primuxime | Profurex | Proroxime | Provixime | Proxim | Quadcef | Qualifur | Quiroxef | Raja | Restum | Retacef | Revacef | Rexocare | Rexofen | Rezafil | Rezafil wfi | Robecim | Robisef | Rocef | Rojacef | Romicef | Rovix | Roxicef | Roxime | Roxiprime | Roxxin | Roxym | Rucef | Ruxim | S rox | Secunda | Sharox | Shincef | Sqcef | Sufrexim | Suncetin | Theoroxime | Titacef | Trigenna | Trixime | Trucef | Unoximed | Verile | Vhersef | Viacef | Vispocef | Vitaroxima | Xefu | Xefustal | Ximgencef | Xinatrene | Xorimax | Xyfrox | Zefcid | Zefcure 500 | Zefcure 750 | Zefsur | Zefur | Zefurox | Zefuxim | Zefuxime | Zegen | Zenoxim | Zicef | Zillian | Zinacare | Zinacef | Zinaf | Zinnat | Zocef | Zoltax | Zurenix | Zurevee | Zyrex;
- (PK) Pakistan: Benroxim | Cefogram | Cefril | Ceftin | Cefucef | Cefurox | Cefuzin | Cegrox | Celoxime | Ceroxime | Dowcef | Evorox | Furains | Furoxi | Infrid | Inrox | Kefacef | Kefrox | Kefzy | Keynet | Libroxin | Lute | Miroxim | Mixorec | Obtrude | Optik | Purox | Puroxime | Roxime | Sayfruxime | Sefur | Sefuspore | Seroxin | Supero | Tiracef | Vegarox | Xurox | Zafurox | Zecef | Zinacef | Zinxef | Zocef;
- (PL) Poland: Aprokam | Biocefal | Biofuroksym | Bioracef | Cefox | Cefuroxim Kabi | Cefuroxim MIP | Cefuroxime 1A Pharma | Cefuroxime axetil aurovitas | Cefuroxime Ceft Limited | Cefuroxime Genoptim | Cefuroximum 123ratio | Ceroxim | Furocef | Plixym | Tacefur | Tarsime | Ximaract | Xorim | Xorimax | Zamur | Zinacef | Zinnat | Zinoxx;
- (PR) Puerto Rico: Ceftin | Kefurox | Zinacef;
- (PT) Portugal: Antibioxime | Aprokam | Cefuroxima | Cefuroxima actavis | Cefuroxima arrowblue | Cefuroxima basi | Cefuroxima Cinfa | Cefuroxima generis | Cefuroxima hikma | Cefuroxima labesfal | Cefuroxima pharmakern | Cefuroxima sandoz | Cefuroxima teva | Curoxime | Zipos | Zoref;
- (PY) Paraguay: Actocef | Axetie | Cefuroxima dutriec | Cefuroxima guayaki | Cefuroxima heisecke | Cefuroxima lasca | Cerevax | Kesint | Zinnat;
- (QA) Qatar: Auroxetil | Axetine (IV/IM) | Cefaks | Cefovex | Cefutil | Daroxime | Maxil | Xorim | Xorimax | Zamur | Zinacef | Zinnat;

- (RO) Romania: Aprokam | Axetine | Axycef | Cefuroxima | Cefuroxima aurobindo | Ceroxim | Kefurox | Lifurox | Zinacef | Zinnat;
- (RU) Russian Federation: Acenoveriz | Aksosef | Antibioxim | Axetine | Cefroksim j | Cefurabol | Cefuroksim | Cefuroxim | Cefuroxim j | Cefuroxim natrii | Cefuroxime | Cefuroxime gphc | Cefuroxime kabi | Cefurus | Cefuxime | Cetil | Kefstar | Ketocef | Proxime | Supero | Velocesim | Xorim | Zinacef | Zinnat | Zinoksimor;
- (SA) Saudi Arabia: Cefofix | Cefovex | Cefrotux | Cefurex | Cefuroxime | Cefuroxime venus | Cefutil | Cefuzime | Ceruxim | Daroxime | Emditil | Maxil | U cef | Xorimax | Zamur | Zenon | Zinacef | Zinnat | Zinoxime | Zinoximor;
- (SE) Sweden: Aprokam | Axacef | Cefuroxim | Cefuroxim fresenius kabi | Cefuroxim norcox | Cefuroxim sandoz | Cefuroxim scand pharm | Cefuroxim stragen | Cefuroxim villerton | Cefuroxime Stravencon | Zinacef | Zinnat;
- (SG) Singapore: Axetine | Cefxin | Shincef | Xorimax | Zinacef | Zinnat;
- (SI) Slovenia: Cefuroksim actavis | Cefuroksim alkaloid int | Cefuroksim aptapharm | Ketocef | Novocef | Tvindal | Zinacef | Zinnat;
- (SK) Slovakia: Aprokam | Axetine | Cefuroxim Kabi | Ceroxim | Furocef | Xelacef | Ximaract | Xorim | Xorimax | Zinacef | Zinnat | Znobact;
- (SL) Sierra Leone: Cifarox | Nirox | Sanfurox | Sefzitil | Zinacef;
- (SR) Suriname: Zinacef | Zinnat;
- (TH) Thailand: C tri t | Cefamar | Cefogen | Ceftil | Cefurim | Cefuroxime | Farmacef | Furoxime | Magnaspor | Neurox | NIKP Cefuroxime | Zinacef | Zinnat | Zocef | Zonef;
- (TN) Tunisia: Aprokam | Cefuroxime | Cefuroxime kabi | U cef | Zifur | Zinnat | Zinox;
- (TR) Turkey: Aksef | Aprokam | Avifur | Cefaks | Cefatin | Cefeye | Cefrox | Ceftop | Cefurol | Enfexia | Inceptum | Multisef | Oraceftin | Powercef | Sefaktil | Seffur | Sefuroks | Tugens | Zannacef | Zinacef | Zinnat;
- (TW) Taiwan: Axurocef | Ceflour | Cefoprim | Cefoxin | Cefuroxime | Cefxin | Cekonin | Cfu | Furoxime | Gibicef | Lafurex | Lifurox | Menimycin | Romicef | Ucefaxim | Uroxime | Zinacef | Zinate | Zinnat;
- (UA) Ukraine: Abitsef farmyunion | Aksef | Auroxetil | Axetine | Bactilem | Biofuroksym | Biofuroxim | Cefumax | Cefurabol | Cefurox | Cefuroxim | Cefuroxime | Cefutil | Cetil | Enfexia | Euroxim | Furexa | Furocef | Kimacef | Micrex | Spizef | Yokel | Zinacef | Zinnat | Zocef;
- (UG) Uganda: Auroxetil | Axetine | Cef2 | Cefakind | Cefrim | Cefrotil | Cefuaxit | Cefuroxime mj | Cefutil | Cefuzime | Celza | Cortum | Daroxime | Furocef | Infuroxim | Kefstar | Proximexa | Pulmocef | Restum | Sanfur | Syner cef | Tryoxim | Tryoxim ds | Zenoxim | Zinacef | Zinaxime | Zinnat | Zocef;
- (UY) Uruguay: Axetie | Cefuroxima | Cefuroxima sandoz | Cefuroxime | Cefuroxime Ion | Cefuroxime ion | Kesint | Roxime | Zefurox | Zinnat | Zinocep;
- (VE) Venezuela, Bolivarian Republic of: Cefuroxima | Seudacin | Xorim | Zencef | Zinacef | Zinnat;
- (VN) Viet Nam: Actixim | Bestnats | Bifumax | Bilvacef | Bio dacef | Cecopha | Cefaxil | Cefcenat | Cefurimaxx | Cefurobiotic | Cefurofast | Cefurovid | Emixorat | Eucinat | Farinceft | Furocap | Glanax | Haginat | Negacef | Nilibac | Noruxime | Oralfuxim | Pulracef | Santorix | Spizef | Undtas | Vanmenol | Zalrinat | Zaniat | Zanimex | Zidunat | Zinextra | Zinnat;
- (ZA) South Africa: Alkoxime | Auroxime | Betaroxime t | Cefasyn | Cefuaxit | Ceroxim | Cipofix | Curoax | Furocsem | Gulf cefuroxime | Lifurom | Oratil | Pharmacare-cefuroxime | Ricifur | Rolab-cefuroxime | Sandoz cefuroxime | Zefroxe | Zefurime | Zinacef | Zinnat | Zinoxime;
- (ZM) Zambia: Altacef | Cef | Cefakind | Cefogen | Cefuaxit | Cefurox | Cefuroxime | Cefuxime | Celza | Ceroxim | Cetil | Ceurox | Kefrox | Restum | Sanfurox | Spizef | Uroxime | Zinacef | Zinnat;
- (ZW) Zimbabwe: Ceroxim | Kefloxa 250 | Kefloxa 500 | Zinacef | Zinnat

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